

Rigid Bronchoscopy Procedure-Specific Checklist (PSF)

Learner: _____

Date: _____

Observer: _____

Case Setting: ☐ OR ☐ Bronch Suite

Indication (check all that apply):

☐ Malignant CAO ☐ Benign stenosis ☐ Foreign body ☐ Hemoptysis ☐ Stent issue ☐ Other:

Interventions Performed (check all that apply):

☐ Diagnostic only ☐ Mechanical debulking ☐ Ablation (laser/APC/cautery/cryo)

☐ Dilation (rigid/balloon) ☐ Stent placement/removal ☐ Foreign body removal

☐ Hemorrhage control ☐ Other: _____

Suggested rating for each item (adapt as needed):

0 = Not done / unsafe | 1 = Needs significant guidance | 2 = Minimal prompting | 3 =

Independent / safe

N/A = Not applicable for this case

A. Pre-Procedure Assessment & Planning

1. Clarifies indication and urgency for rigid bronchoscopy. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Reviews relevant imaging and prior airway interventions; articulates procedural plan. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Screens for contraindications (cervical spine instability, hemodynamic instability, difficult anatomy). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Coordinates with anesthesia regarding **TIVA**, neuromuscular blockade, and ventilation strategy. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
5. Addresses anticoagulation/antiplatelet status and relevant labs; confirms consent and NPO status. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
6. Leads or participates in a structured pre-procedure briefing / timeout (roles, plan, anticipated challenges, backup plans). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

B. Equipment & Room Setup

1. Selects appropriate rigid bronchoscope size based on patient habitus and pathology. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 2. Assembles rigid scope, telescope, and light source; confirms all function properly. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 3. Ensures ventilation setup is appropriate (controlled, spontaneous-assisted, or jet) with ETCO₂/monitoring as feasible. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 4. Confirms adequate suction (ideally two systems) and appropriate catheters available. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 5. Ensures availability of necessary instruments for planned intervention (forceps, balloons, ablation devices, stents, foreign body tools, cryoprobes). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 6. Completes safety checks: dental guard, energy/laser safety (FiO₂ limits, eye protection), hemorrhage and emergency airway equipment. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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C. Intubation & Airway Control

1. Positions patient optimally (e.g., sniffing position or appropriate modification if cervical spine issues). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 2. Confirms adequate anesthetic depth and paralysis before intubation attempt. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 3. Introduces scope with appropriate mouth and dental protection; avoids trauma to lips/teeth. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 4. Identifies epiglottis and vocal cords reliably; uses epiglottis elevation technique appropriately. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 5. Performs smooth 90° rotation and passes through vocal cords without undue force; stops and reassesses when resistance encountered. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 6. Confirms tracheal placement with visualization of tracheal rings and carina; stabilizes scope and establishes ventilation. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 7. Adapts intubation strategy for difficult airway (use of laryngoscope, smaller scope, alternative approaches) when needed. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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D. Systematic Airway Inspection

1. Performs systematic airway survey (trachea → carina → main bronchi ± lobar bronchi) using telescope or flexible scope as needed. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 2. Accurately describes location, length, and type of pathology (intrinsic vs extrinsic, stenosis type, tumor morphology, bleeding). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 3. Correlates bronchoscopic findings with imaging and refines procedural plan accordingly. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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E. Therapeutic Technique (mark N/A if not part of this case)

1. Mechanical Debulking

1. Plans debulking strategy (scope coring vs forceps) to restore lumen while preserving normal tissue. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Uses coring technique effectively (controlled forward pressure and rotation, avoids excessive force). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Uses forceps appropriately (instrument selection, bite–twist–withdraw technique, maintains visualization). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Manages bleeding during debulking (patient positioning, suction, iced saline, tamponade, energy hemostasis as indicated). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

2. Ablation (Laser / APC / Electrocautery / Cryotherapy)

1. Confirms FiO₂ is at or below target (e.g., ≤40%) and ventilation mode is appropriate before activating energy. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Chooses ablation modality and settings appropriate for lesion type and depth. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Uses safe technique (appropriate distance, pulse duration, avoids firing on metal stents or dry surfaces). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Achieves desired tissue effect while maintaining airway patency and minimizing collateral injury. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

3. Dilation (Rigid and/or Balloon)

1. Measures/estimates stenosis diameter and length; selects appropriate rigid scope and/or balloon size. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

2. Performs graded dilation (incremental size increases; adheres to safe “rule of 3s” principles). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Monitors resistance, patient status, and stops appropriately when warning signs appear. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Recognizes and responds to complications (mucosal tear, bleeding, subcutaneous emphysema/suspected perforation). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

4. Stent Placement / Management

1. Selects appropriate stent type, diameter, and length based on pathology (benign vs malignant, straight vs Y). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Loads stent correctly into applicator with proper orientation and adequate lubrication. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Deploys stent under direct visualization (distal end first, then proximal) while maintaining accurate positioning. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Confirms stent expansion, coverage of pathology, and absence of significant obstruction of adjacent segments. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
5. Recognizes and manages immediate stent complications (migration, kinking, incomplete expansion). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

5. Foreign Body Removal

1. Selects retrieval device appropriate to foreign body type (organic, metallic, tooth/bone, plastic, etc.). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Keeps foreign body in view and avoids pushing it distally during manipulation. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Coordinates extraction with anesthesia/ventilation to minimize risk of loss or aspiration. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
4. Performs final inspection to confirm no residual fragments or airway injury. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

F. Monitoring, Safety, and Crisis Management

1. Maintains continuous awareness of SpO₂, hemodynamics, and ventilation (including ETCO₂ when available). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

2. Communicates clearly with anesthesia regarding FiO₂ changes, ventilation mode, and procedural steps that affect oxygenation. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 3. Recognizes early signs of hypoxemia, hypercapnia, or hemodynamic instability and pauses/restructures procedure appropriately. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 4. Recognizes major complications promptly (significant hemorrhage, suspected pneumothorax, airway fire) and initiates appropriate protocol (e.g., tamponade and iced saline, stopping energy and O₂, calling for help). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 5. Delegates tasks effectively during crisis (assigns roles, uses closed-loop communication, involves additional resources). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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G. Post-Procedure Management

1. Performs final airway inspection; confirms patency, stent position (if present), and adequate clearance of secretions/clots. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 2. Coordinates emergence/extubation strategy with anesthesia, including plans for post-procedure airway support as needed. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 3. Provides appropriate post-procedure orders (monitoring level, oxygen, bronchodilators, nebulized saline/mucolytics, imaging when indicated). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 4. Communicates results, interventions, and contingency plans to the receiving team and, when appropriate, to patient/family. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
 5. Documents procedure clearly, including indication, techniques used, devices placed/removed, complications, and follow-up plan. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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H. Non-Technical Skills / Teaching & Leadership

1. Demonstrates clear, calm communication with the team throughout the case. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
2. Shows situational awareness (anticipates next steps, asks for equipment in advance, recognizes evolving risk). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
3. Creates a psychologically safe environment (invites input, responds well to concerns, avoids blame). ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A

4. Provides brief, targeted teaching to junior learners when appropriate without compromising safety. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
5. Participates in post-case debrief, identifies successes and areas for improvement.
☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ N/A
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I. Global Rating / Entrustment

Overall performance on this case:

- ☐ 0 – Unsafe; cannot perform any part without direct hands-on supervision
- ☐ 1 – Performs portions of the procedure with continuous, directive supervision
- ☐ 2 – Performs most steps with minimal verbal guidance; supervisor immediately available
- ☐ 3 – Independently performs complete rigid bronchoscopy for this indication, including management of anticipated complications; able to supervise junior learners

Areas of particular strength:

Priority areas for further practice (targeted goals):
